Strengthening the intersectoral response provided to the different concurrent social and healthcare needs of older people living at home: profiles, resource mapping and network analysis.



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01 Context

CCSMTL: 8 CLSC territories, 48,575 people aged 65 and over in 2026
ISQ, 2026

≈ 3,700
requests to 211 from people aged 70 and over, 2021–2026
211 Grand Montréal, 2026

691
unmet needs, aged 60 and over (sensitivity analysis)
211 Grand Montréal, 2026

THREE DISTINCT NOTIONS

Older adults followed at home who face several social needs; a response organized within the CLSC territory.

Concurrent needs

Social and healthcare needs occurring in the same people.

Older people living at home

older people living at home with concurrent social and healthcare needs that may require an intersectoral response in primary care.

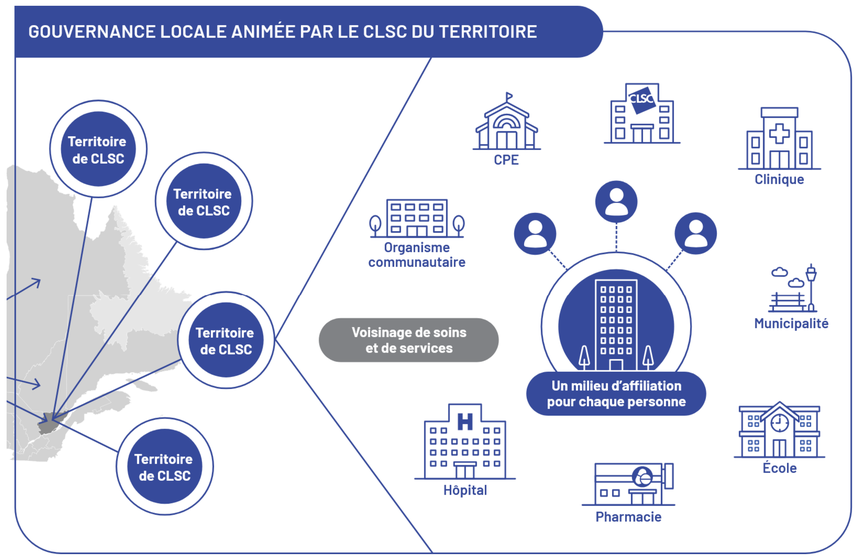
ORGANIZED WITHIN A TERRITORY

Intersectoral response

Actions by sectors other than health, carried out with it, on health or its determinants: public network, primary care, parapublic, community, municipality, housing.
MSSS, 2017; Leppo et al., 2013

QUÉBEC PRIMARY CARE POLICY · MSSS, 2026

The CLSC territory becomes the reference unit



FOUR ROLES OF THE CLSC

Navigator · Coordinator · Intersectoral leader · Co-facilitator

"in a spirit of shared governance" · MSSS, 2026, p. 42 · figure: MSSS, 2026 (in French)

How is the intersectoral response to concurrent social and healthcare needs organized for different profiles of older people living at home?

O1
Identify different profiles of older people living at home with concurrent social and healthcare needs that may require an intersectoral response in primary care.

O2
Map territorial resources that may be mobilized in primary care to address the concurrent social and healthcare needs of different profiles of older people living at home, and characterize territorial variations in their availability, diversity, complementarity and accessibility.

O3
Analyze the organizational configurations of intersectoral networks in primary care mobilized across the territory to address the concurrent social and healthcare needs of different profiles of older people living at home, in order to characterize how they operate.

02 Proposed methods

PROVISIONAL PROTOCOL · sites, partnerships and approvals to be confirmed

older people living at home with concurrent social and healthcare needs that may require an intersectoral response in primary care.

Methodological objective O1

Conduct a review of the scientific literature to identify and characterize different profiles of older people living at home with concurrent social and healthcare needs that may require an intersectoral response in primary care, and to determine the needs associated with each of these profiles.

Scoping review · 4 databases

- systematic searches, explicit eligibility criteria and two independent reviewers.
- populations, primary-care context, social and healthcare needs, definitions, instruments, co-occurrence in the same people and reported profiles.
- JBI scoping review, reported with PRISMA-ScR; descriptive synthesis of definitions, associated needs and documented configurations.

Methodological objective O2

Conduct a cartographic and spatial study of resources that can be mobilized in primary care to address the different concurrent social and healthcare needs associated with the profiles of older people living at home, in order to characterize their distribution, availability, diversity, complementarity and territorial accessibility across the different CLSC territories.

6 sectors · ecological analysis

- systematic identification of eligible resources in selected sources; explicit inclusion and deduplication rules.
- location, sector, relevant needs, documented availability, access conditions, eligibility, opening hours and capacity when available.
- cartographic and spatial study in R and QGIS; distribution, availability, diversity, complementarity and territorial accessibility.

Methodological objective O3

Conduct a retrospective social network analysis (SNA) of intersectoral networks mobilized in primary care from CLSC territories to address the concurrent social and healthcare needs of different profiles of older people living at home, in order to characterize their composition, the structure of their relationships, and the nature and intensity of collaboration between actors.

One questionnaire · 12 months · whole and ego networks

- one relational questionnaire administered once to organizational actors about the previous 12 months.
- ties among all organizations within each territorial boundary; composition, structure and operating arrangements, subject to coverage.
- partners of focal organizations or teams, including the CLSC; size, diversity and ego-alter ties. Alter-alter ties require explicit collection.

O1: profiles · O2: resources · O3: networks. Retrospective quantitative observational SNA across several contrasting CLSC territories.

03 Expected outputs · no thesis findings presented

Supported profiles + vignettes

Atlas + needs-resources-territory matrix

Contextualised networks + mechanisms

04 Intended contribution

Inform gaps between needs, accessible provision and mobilised help; discuss organisational options with the people concerned and territorial stakeholders. No individual benefit is guaranteed.

Contact ≠ collaboration ≠ help received ≠ need resolved. The 211 figures and contextual maps are illustrative; they do not replace O3 data collection.

Brewster, Tan, & Yuan (2019). Health Services Research, 54(6), 1246-1254. doi:10.1111/1475-6773.13206 · AHRQ (2014), Care Coordination Measures Atlas, ch. 4.